RADINTEL AI / RADIOLOGY ONLY

Business Plan

Commercial strategy, pricing hypotheses, costs, market validation and risk management

Version 1.0 | 14 October 2026 | Planning draft - not clinical or legal advice

1. Executive summary

RadIntel AI is a radiology-specific B2B software concept for Kosovo diagnostic clinics. Initial value proposition: faster physician-reviewed reporting, more organized examination worklists, easier access to historical studies, and auditable delivery. The platform is not an autonomous diagnostic tool. Launch via focused private-clinic pilots and expand to additional facilities only after measurable validation.

2. Market problem and timing

Kosovo is modernizing health information infrastructure through the KOMPAS/IHIS program. This creates a rationale for standards-compatible radiology workflow tools, but also raises integration and procurement dependencies. No defensible total addressable market, clinic count, or willingness-to-pay figure is asserted without field research.

3. Customer segmentation

Segment	Buyer	Need	Sales consideration
Small private imaging clinic	Owner / lead radiologist	Reporting, worklist, scheduling	Fast setup and predictable price
Multi-site diagnostic center	Operations director	Standardized reporting and dashboards	Tenant/facility configuration
Hospital radiology department	Hospital IT / procurement	PACS/RIS integration, security	Long procurement and integration cycle
Teleradiology group	Clinical director	Distributed assignments, report delivery	Cross-site governance and audit

4. Value proposition and differentiation

- Albanian-first and English-supported report templates, validated with local radiologists.

- Integrate existing PACS rather than requiring replacement of medical image infrastructure.
- Clinical workflow ownership: versioned reports, sign-off, addenda, role-scoped access.
- Flexible private-cloud or on-prem deployment where feasible and contracted.
- AI positioned as an optional assistive tool with explicit doctor control.

5. Competitive landscape - research plan

Map each prospective customer existing RIS, PACS, EHR, dictation, scheduling, and outsourced radiology services. Compare vendor pricing, contract lock-in, language support, interoperability, training burden, and security posture. Do not assume no local competitors. Request 5-10 structured vendor/customer demonstrations.

6. Product packaging and pricing hypotheses

Package	Indicative monthly price	Included / limits
Starter	EUR 79	1 facility; worklist, templates, reporting; capped users
Clinic	EUR 249	Scheduling, audit, basic PACS integration, more users
Professional	EUR 599	Multi-facility dashboards, dictation/AI allowance, priority support
Enterprise	Custom quote	On-prem, integrations, SLA, implementation and security review

Prices above are test assumptions, not market quotes. Exclude storage-intensive imaging archive charges and usage-based AI from base packages unless unit economics permit. Setup and integration fees may be needed.

7. Go-to-market roadmap

Period	Activities	Exit criteria
Weeks 1-4	Interview 10-15 clinics/radiologists; map workflows; validate regulatory scope	2 design partners; signed problem statements
Weeks 5-8	Prototype reporting/worklist with synthetic data; usability testing	Clinician workflow approval
Months 3-5	MVP build; security and DICOM integration testing	Technical and clinical readiness checklist
Months 6-7	1-2 supervised pilots with defined KPIs	Documented value and incident-free operating evidence
Months 8-12	Paid conversion, reference case studies, support process	At least 2 paying customers; measurable retention

8. Unit economics assumptions

Illustrative scenario only: variable operating cost EUR 35 per Starter clinic, EUR 80 per Clinic, and EUR 180 per Professional customer per month; excludes shared engineering, sales, compliance and acquisition costs. Contribution margins before fixed costs: EUR 44, EUR 169, and EUR 419 respectively. Validate AI token usage, storage, backup, support and payment costs before launch.

9. Illustrative recurring-revenue scenarios

Scenario	Customer mix	Monthly revenue	Annualized run rate
Conservative	5 Starter + 3 Clinic + 1 Professional	EUR 1,741	EUR 20,892
Base	10 Starter + 8 Clinic + 3 Professional	EUR 4,579	EUR 54,948
Growth	20 Starter + 15 Clinic + 6 Professional	EUR 8,909	EUR 106,908

Annualized run rate = one month recurring revenue x 12; not recognized revenue, profit, or forecast. Assumes all customers pay list price with no churn or discounts.

10. Startup budget - illustrative planning range

Cost category	One-time EUR	Monthly EUR
Discovery and clinical advisor	2,000-6,000	300-1,000
MVP engineering (founder-led or contracted)	8,000-35,000	Variable
Security/legal/privacy preparation	2,000-10,000	100-500
Hosting, backup, monitoring, AI	500-2,000	150-900
Pilot onboarding/training	1,000-4,000	100-400
Sales/support operations	500-2,000	300-1,500

These are directional planning estimates, not supplier quotes. A founder doing most engineering may have lower cash expense but significant opportunity cost. Clinical-grade imaging and regulated AI would materially increase costs.

11. Key risks and mitigations

Risk	Impact	Mitigation
Clinics already have sufficient software	Weak product-market fit	Interview and observe before building
PACS integration variability	Slow onboarding	Adapters, integration matrix, paid discovery
Medical data breach	Severe patient and legal harm	Least privilege, encryption, testing, response plan
AI hallucination	Clinical safety and trust	Draft-only, provenance, human sign-off
Regulatory misclassification	Launch delay	Legal/regulatory assessment per intended use
Low willingness to pay	Poor unit economics	Paid pilots, modular packaging, cost limits
National system integration changes	Rework	FHIR/DICOM standards and adapter isolation

12. Decision gates and metrics

- Gate A - discovery: at least 10 interviews and 2 clinics willing to pilot.
- Gate B - prototype: radiologists confirm usability and correct report semantics.
- Gate C - launch: legal/privacy assessment, security review, signed pilot contract and incident plan.
- Gate D - scale: paid conversion, stable support load, positive contribution margin, measured efficiency benefit.

13. Legal and commercial diligence

Confirm Kosovo personal data protection requirements, health data processing roles, professional liability, data-hosting restrictions, medical-device software classification based on intended use, vendor contracts, cyber insurance, and procurement eligibility. GDPR may also matter when processing EU residents data or serving EU customers; applicability must be evaluated, not assumed.

References and evidence boundaries

World Bank, Kosovo healthcare strengthening (2024) — https://www.worldbank.org/en/news/press-release/2024/05/07/world-bank-supports-enhancement-of-health-services-in-kosovo-improving-the-lives-of-its-people

Kosovo Ministry of Health, IHIS oversight procurement (2026) — https://msh.rks-gov.net/Documents/Index/1?year=2026

World Bank, Kosovo Digital Health Blueprint case study — https://thedocs.worldbank.org/en/doc/b929829fc3d36bebdf9d93555c64f222-0140022024/related/Digital-Health-Blueprint-Toolkit-Kosovo-Case-Study.pdf

HL7 FHIR specification — https://hl7.org/fhir/

OHIF documentation — https://docs.ohif.org/

OHIF DICOMweb and Orthanc integration — https://docs.ohif.org/configuration/datasources/dicom-web/

Market demand, exact waiting times, clinic counts, current vendor penetration, willingness to pay, and legal classification require local primary research. All numerical commercial scenarios in this document are illustrative assumptions.